PROGRAM MANAGEMENT · HEALTHCARE

Program command center

A hospital EHR modernization, seen through the dashboard: plan, risk, budget, delivery, transformation and what-if.

Fictional data · Meridian Health Network · 5 hospitals · 18 months · $14M

Swipe ›

Steering committee view

Read in 60 seconds. Overall status is the worst-of five dimensions, with every threshold visible in the About tab.

OVERALL PROGRAM HEALTH

▲ AT RISK

WAVE 1 GO-LIVE
Jan 26, 2027

PROGRAM COMPLETION
Aug 31, 2027

SCHEDULE
On plan
▲ At risk

Wave 1 go-live vs baseline. Tightest float: 1 days on 3 milestones.

BUDGET
$15.0M
▲ +7.2% forecast

Baseline $14.0M. Spent $6.3M vs $5.9M planned.

RISK EXPOSURE
6 high
▲ At risk

Open items scoring 15+ of 25. 4 flagged for escalation.

BENEFITS
85%
▲ At risk

Realized vs planned to date, averaged over tracked benefits.

CLINICIAN READINESS
64%
▲ At risk

Average site readiness. Target 80% before each go-live.

Decisions needed from the committee

1. **Release $0.8M of contingency** to cover the forecast overrun of $1.0M; remaining $0.3M offset by deferring 14 order-set requests.
2. **Approve 4 contract trainers** (about $0.4M) to close the Dec to Mar training gap, protecting the Wave 1 date.
3. **Authorize an executive escalation** to the pharmacy interface vendor under the SLA clause (R-01, score 20).

Top risks and issues

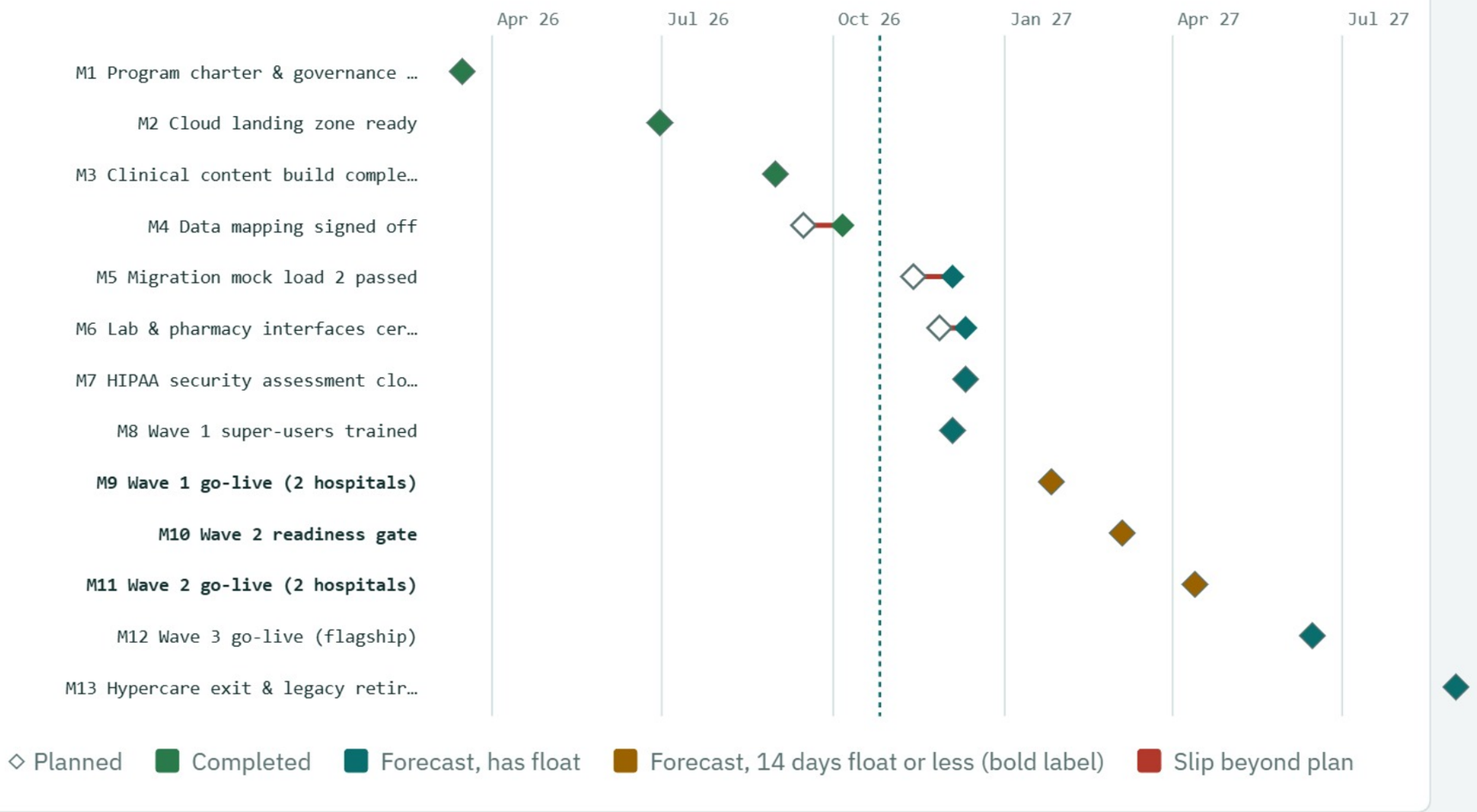
R-01	Pharmacy interface vendor delivers certified build late Weekly vendor exec check-in; manual order fallback designed; SLA penalty clause invoked.	20
I-01	Data mapping sign-off landed 3 weeks late Mock load 2 re-planned; extra data engineers loaned from Cloud Platform.	20
R-02	Training capacity gap of 260 nurse hours/week, Dec to Mar Contract 4 trainers; shift to 4-hour modular sessions; protect super-user time.	16

Workstreams

WORKSTREAM	LEAD	STATUS	COMPLETE	NOTE
Clinical Workflow	Dr. A. Rao	▲ At risk	62%	Order-set requests up 14; scope control in place.
Data Migration	M. Fischer	■ Off track	48%	Mock load 2 forecast 3 weeks late; duplicate MRNs at 3.1%.
Integrations	P. Nair	▲ At risk	55%	Pharmacy interface vendor build 2 weeks behind.
Infrastructure & Cloud	L. Okafor	● On track	85%	Landing zone live; network upgrade at Harborview due Dec 5.
Training & Change	S. Brandt	▲ At risk	40%	260 nurse-training hours per week short Dec to Mar.
Compliance	J. Whitfield	● On track	58%	Audit-logging finding being remediated.

Program plan

Planned versus forecast dates. Forecast follows the dependency chain: a milestone cannot finish earlier than its slowest predecessor plus the minimum lag.



Thin-float chain

These milestones have 14 days of float or less. A slip upstream moves them directly.

M9 Wave 1 go-live (2 hospitals)
float 1 days · forecast Jan 26, 2027 · waits on M5, M6, M7, M8

M10 Wave 2 readiness gate
float 3 days · forecast Mar 5, 2027 · waits on M9

M11 Wave 2 go-live (2 hospitals)
float 9 days · forecast Apr 13, 2027 · waits on M10

Dependency table

MILESTONE	DEPENDS ON	PLAN	FORECAST
M5 Migration mock load 2 passed	M4	Nov 13, 2026	Dec 4, 2026
M6 Lab & pharmacy interfaces certified	M2	Nov 27, 2026	Dec 11, 2026
M7 HIPAA security assessment closed	M2	Dec 11, 2026	Dec 11, 2026
M8 Wave 1 super-users trained	M3	Dec 4, 2026	Dec 4, 2026
M9 Wave 1 go-live (2 hospitals)	M5, M6, M7, M8	Jan 26, 2027	Jan 26, 2027
M10 Wave 2 readiness gate	M9	Mar 5, 2027	Mar 5, 2027
M11 Wave 2 go-live (2 hospitals)	M10	Apr 13, 2027	Apr 13, 2027
M12 Wave 3 go-live (flagship)	M11	Jun 15, 2027	Jun 15, 2027
M13 Hypercare exit & legacy retired	M12	Aug 31, 2027	Aug 31, 2027

RAID log

Score is probability × impact, each 1 to 5. Escalate when an open item scores 16 or more, or has aged past 45 days with a score of 12 or more.

All types | All workstreams | Open only | Search title or owner

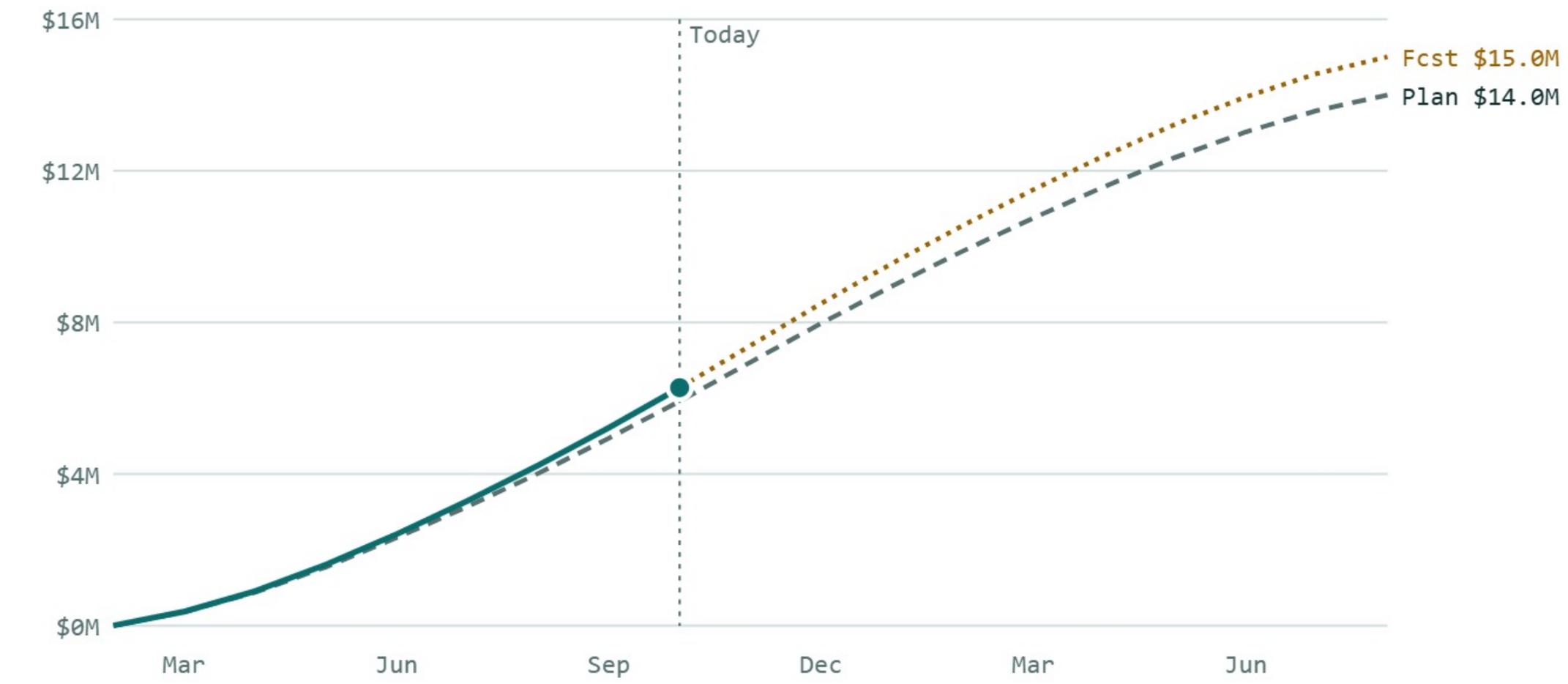
Top 7 open items by score

ID	TYPE	ITEM	OWNER	SCORE	AGE	FLAG
R-01	Risk	Pharmacy interface vendor delivers certified build late Integrations · Weekly vendor exec check-in; manual order fallback designed; SLA penalty clause invoked.	P. Nair	■ 20	69 d	▲ Escalate
I-01	Issue	Data mapping sign-off landed 3 weeks late Data Migration · Mock load 2 re-planned; extra data engineers loaned from Cloud Platform.	M. Fischer	■ 20	41 d	▲ Escalate
R-02	Risk	Training capacity gap of 260 nurse hours/week, Dec to Mar Training & Change · Contract 4 trainers; shift to 4-hour modular sessions; protect super-user time.	S. Brandt	■ 16	54 d	▲ Escalate
R-03	Risk	Duplicate patient records (3.1%) fail mock load 3 Data Migration · Probabilistic matching sprint; HIM team clears top 8,000 duplicates before Nov 30.	M. Fischer	■ 15	35 d	
I-02	Issue	Three super-user roles vacant at Riverbend Training & Change · Backfill requests approved by CNO; interim cover from Northgate.	S. Brandt	■ 15	25 d	
I-03	Issue	Forecast at completion is 7% above baseline Program Office · Contingency draw request in steering pack; scope trade-offs listed.	Program Director	■ 15	13 d	
R-04	Risk	Wave 1 cutover overlaps winter flu surge Clinical Workflow · Move cutover weekend to Jan 23; add 20% hypercare floor-walkers.	Dr. A. Rao	▲ 12	26 d	

Budget and capacity

Cumulative spend against plan, and team demand against available capacity for the next six months.

Cumulative spend



Spend runs about 8% above plan in each month since April. The forecast assumes that rate holds.

1. View as table

MONTH	PLAN CUM.	ACTUAL/FCST CUM.

Forecast at completion

Baseline budget	$14.0M
Spent to date	$6.3M
Planned to date	$5.9M
Variance to date	+$0.4M
Forecast at completion	**$15.0M**
Overrun vs baseline	+$1.0M (7.2%)
Contingency held	$0.8M

Capacity heatmap: demand as % of capacity

	NOV	DEC	JAN	FEB	MAR	APR	TEAM SIZE
Clinical Informatics	96%	98%	104%	108%	102%	95%	10
Data Engineering	118% ▲	112% ▲	104%	96%	90%	88%	8
Integration Engineering	108%	112% ▲	106%	98%	94%	90%	7
Cloud Platform	84%	80%	78%	76%	72%	70%	6
Training Team	104%	121% ▲	126% ▲	124% ▲	118% ▲	108%	9
Security & Compliance	92%	100%	96%	88%	82%	80%	5
Change & Program Office	90%	94%	96%	98%	92%	90%	4
QA & Test	100%	106%	112% ▲	114% ▲	104%	96%	8

85% or less · 86 to 100% · 101 to 110% · Over 110% ▲ overallocated

Training and Data Engineering carry the peaks. Cloud Platform runs at 70% at lowest, so loaning two engineers to Data is feasible.

Delivery health

Predictability and quality for the build teams feeding Wave 1. Say/do compares completed with committed points.

SAY/DO RATIO

88%

▲ At risk

Six-sprint average. Target 90%.

DEFECT LEAKAGE

6.2%

▲ At risk

Defects found in UAT that escaped system test. Target 5%.

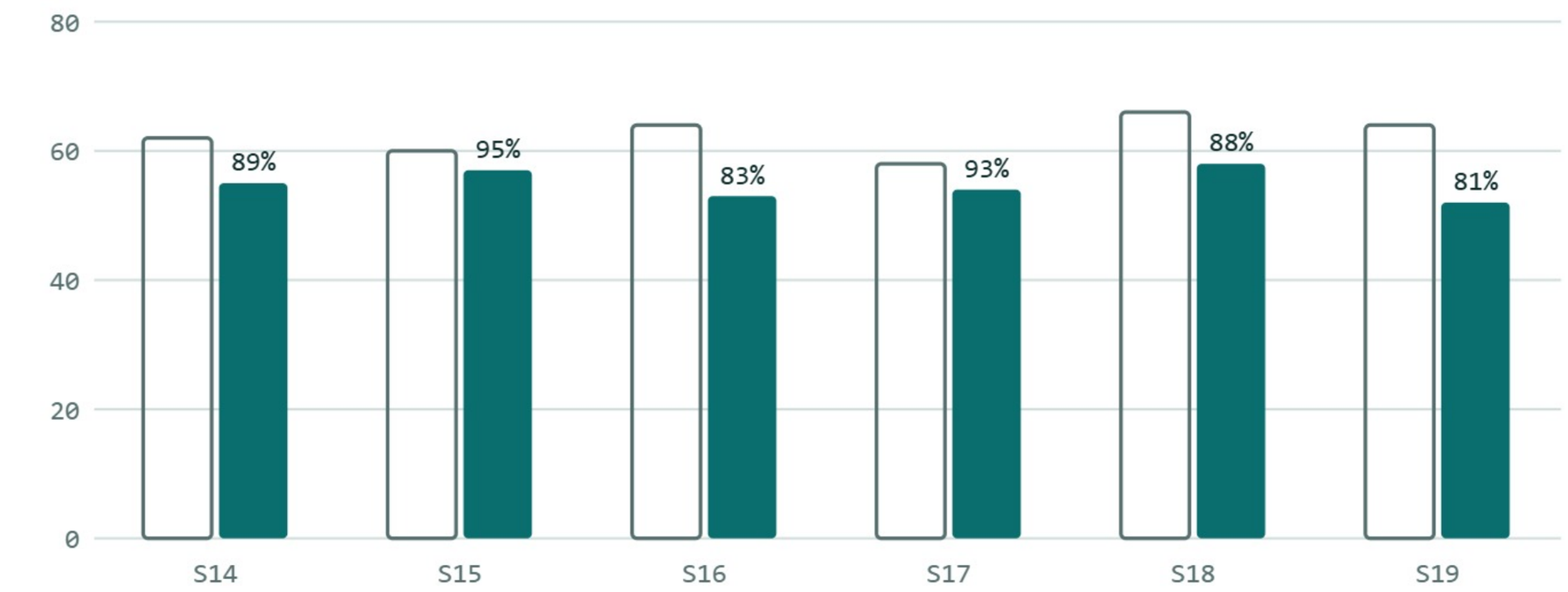
LEAD TIME, CONFIG CHANGE

9 days

● On track

Request to production, down from 16 in Q2.

Committed vs completed points



Wave 1 release readiness

Test automation coverage	58% vs 70% target	▲ At risk
Sev 1 defects open	0	● On track
Sev 2 defects open	11 (limit 15)	● On track
Performance test at 1.5× peak load	Not yet run	■ Off track
Downtime procedures rehearsed	2 of 5 sites	▲ At risk
Cutover runbook v1 reviewed	Complete	● On track

Transformation

Clinical adoption, benefits and the objectives the program exists to deliver.

Site readiness

SITE	WAVE	READINESS	SUPER-USERS
Northgate General	1	82%	90%
St. Aurelia	1	74%	78%
Riverbend Community	2	58%	55%
Harborview	2	61%	60%
Meridian Central (flagship)	3	44%	35%

Readiness blends leader sponsorship, training completion, workflow sign-off and super-user coverage. Target 80% before go-live.

Benefits realization

BENEFIT	TARGET	PLAN NOW	ACTUAL
Legacy data-centre cost avoided	1.8 $M/yr	0.5	0.4
Duplicate lab orders avoided	18 %	2	2
Pilot clinics on new documentation workflow	12 clinics	4	3
Clinician documentation time saved	25 min/shift	0	0
Claim denial reduction	12 %	0	0

Most clinical benefits start at go-live. Until then the program tracks enablers.

Objectives and key results

O1 Safer, faster care for patients

- KR1 Cut duplicate lab orders by 18% · on track
- KR2 Reduce discharge-summary delay from 2.1 days to 1 day · starts after Wave 1

O2 Give clinicians time back

- KR3 Save 25 minutes per shift in documentation · pilot clinics at 3 of 4 planned
- KR4 80%+ readiness at every site before go-live · at risk: flagship at 44%

O3 Retire legacy systems and cost

- KR5 Decommission 3 data centres, saving $1.8M a year · cloud landing zone live
- KR6 Zero HIPAA findings above medium at assessment · one logging finding open

What-if planner

Test a delay or a staffing change and see it move through the dependency chain. Cost and benefit figures are indicative, using a $0.51M monthly carrying cost and $0.7M monthly benefit value.

Scenario inputs

Slip this milestone

M5 Migration mock load 2 passed

Extra delay: **4 weeks**

4

Remove people from this team

Training Team (9 people)

People removed: **2**

2

Reset scenario

Impact

Wave 1 go-live	Feb 15, 2027	+20 d
Wave 2 go-live	Apr 21, 2027	+8 d
Program completion	Aug 31, 2027	0 d
Added carrying cost	$0.0M	
Benefits pushed out	$0.5M	
Forecast at completion	**$15.0M**	+7.2% vs plan

Removing 2 of 9 from Training Team delays M8 by 13 days.

Milestones that move

MILESTONE	BASELINE FORECAST	SCENARIO	CHANGE
M5 Migration mock load 2 passed	Dec 4, 2026	Jan 1, 2027	+28 d
M8 Wave 1 super-users trained	Dec 4, 2026	Dec 17, 2026	+13 d
M9 Wave 1 go-live (2 hospitals)	Jan 26, 2027	Feb 15, 2027	+20 d
M10 Wave 2 readiness gate	Mar 5, 2027	Mar 22, 2027	+17 d
M11 Wave 2 go-live (2 hospitals)	Apr 13, 2027	Apr 21, 2027	+8 d

Weekly status report

Built from the data in this program, including any active what-if scenario. Change the data and regenerate to update it.

Generate report | Copy text | Download PDF | Report generated at 4:54:54 PM

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MERIDIAN HEALTH NETWORK · EHR MODERNIZATION
Weekly status report · as of Oct 26, 2026

OVERALL: AMBER  (month 8 of 18)

*** INCLUDES WHAT-IF SCENARIO: M5 slips 4 wk; 2 removed from Training Team ***

HEADLINE
Wave 1 go-live is forecast for Feb 15, 2027 (+20 days vs baseline forecast). The Wave 1 to
Wave 2 chain has 1 days of float, so any further upstream slip moves Wave 2.

STATUS BY DIMENSION
  Schedule   AMBER  Data migration mock load 2 is 21 days late; absorbed by float for now.
  Budget     AMBER  Forecast $15.0M vs $14.0M baseline (+7.2%). Spent $6.3M vs $5.9M
planned.
  Risk       AMBER  6 open items scoring 15+; 4 flagged for escalation.
  Benefits   AMBER  85% of planned benefits realized to date.
  Readiness  AMBER  Average site readiness 64% vs 80% target; flagship at 44%.

PROGRESS THIS PERIOD
  - Data mapping signed off (3 weeks late); mock load 2 re-planned.
  - Cloud landing zone live; security review finding closed.
  - Super-user training began at Northgate General (90% complete).

TOP RISKS AND ISSUES
  R-01 (20) Pharmacy interface vendor delivers certified build late
      Mitigation: Weekly vendor exec check-in; manual order fallback designed; SLA penalty
clause invoked.
  I-01 (20) Data mapping sign-off landed 3 weeks late
      Mitigation: Mock load 2 re-planned; extra data engineers loaned from Cloud Platform.
  R-02 (16) Training capacity gap of 260 nurse hours/week, Dec to Mar
      Mitigation: Contract 4 trainers; shift to 4-hour modular sessions; protect super-user
time.

DECISIONS NEEDED
  1. Release $0.8M of contingency toward the $1.0M forecast overrun.
  2. Approve 4 contract trainers (about $0.4M) to close the Dec to Mar training gap.
  3. Authorize executive escalation to the pharmacy interface vendor (R-01).

SCENARIO IMPACT
  Program completion Aug 31, 2027 (0 d); added carrying cost $0.0M; benefits pushed out
$0.5M.

NEXT PERIOD
  - Mock load 2 execution; duplicate-record clean-up sprint.
  - Pharmacy interface certification retest.
  - Union consultation meeting (Nov 4).
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